

NIC CPD Micro-Credential: Wound Care & Basic Dressing Support for Professional Caregivers

Issuing organisation: National Institute of Caregivers Nigeria (NIC)

Credential type: Stand-alone Continuing Professional Development (CPD) Micro-Credential

Online duration: 5–6 CPD learning hours

Practical component: Separate supervised competency verification where the learner's role, training, care plan and employer policy permit dressing support

Version: 1.0 | **Prepared:** August 2026

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Essential scope boundary: A caregiver may support wound care within an authorised role, but an online CPD certificate does not independently authorise a caregiver to diagnose, prescribe treatment, select advanced wound therapies, debride, suture or perform other procedures outside scope.

1. Course title

NIC CPD Micro-Credential: Wound Care & Basic Dressing Support for Professional Caregivers

2. Course overview

This practical CPD introduces wound biology, common wound types, safe observation, infection prevention, dressing categories, basic dressing support, pressure-injury awareness, diabetic-foot awareness, nutrition and hydration links, pain and dignity, complications, documentation and escalation.

It repeatedly distinguishes **wound-care knowledge** from **practical competency**. The online course can teach principles and judgement. Clinical dressing changes, wound measurement, cleansing, product selection and other procedures require current training, authorised delegation, supervision and the client's care plan.

The content is informed by WHO wound-care and infection-prevention principles, international pressure-injury guidance, diabetic-foot guidance and recognised wound-infection resources [1](#) [2](#) [3](#) [4](#) . International guidance must be adapted to the Nigerian facility, employer policy, available supplies and authorised referral pathway.

3. Target audience

Professional caregivers, healthcare assistants, nursing assistants, home-care workers, residential-care staff, hospital support workers, community caregivers, care workers supporting older adults or people with chronic conditions and existing caregivers seeking CPD.

4. Entry expectations

Learners should understand basic caregiving, communication, infection prevention, dignity, documentation and escalation. No clinical qualification is required for the online pathway. Practical verification requires an appropriate assessor, equipment, local protocol and written scope.

5. CPD learning hours

The online component is designed for **5–6 CPD learning hours**, including lessons, illustrations, knowledge checks, scenarios, module assessments, final assessment and job aids.

6. Central competency

Recognise, observe and support safe basic wound care within an authorised scope of practice while protecting wound integrity, preventing infection, promoting comfort and dignity, documenting care accurately, and escalating signs of deterioration appropriately.

7. Learning outcomes

By completion, learners should be able to:

1. Define a wound and explain normal healing phases.
2. Differentiate common acute, chronic, surgical, traumatic, pressure-related, diabetic-related and skin-tear wounds.
3. Recognise factors that delay healing.
4. Explain caregiver role, delegation, supervision and scope.
5. Observe wounds without diagnosing.
6. Explain infection prevention and clean, aseptic and sterile principles.
7. Identify common equipment and dressing categories.

8. Support basic dressing care only within the care plan and authorisation.
9. Recognise infection, pressure injury, poor circulation and delayed-healing warning signs.
10. Link nutrition, hydration and healing without prescribing diet or supplements.
11. Communicate about wounds with dignity, privacy and cultural sensitivity.
12. Recognise pain, fear, odour, dependence and chronic-wound distress.
13. Dispose of contaminated materials safely.
14. Document wound observations, care, changes and escalation accurately.
15. Recognise urgent red flags and refer appropriately.
16. Explain why caregivers must not independently diagnose or prescribe wound treatment.

8. Course structure

Module	Title	Lessons	Assessment
1	Foundations of Wounds and Healing	1. What Is a Wound?; 2. Wound Types; 3. Healing and Delayed Healing	6 questions
2	Wound Observation and Assessment Awareness	4. Observation Versus Diagnosis; 5. What to Observe; 6. Trends and Care Plans	6 questions
3	Infection Prevention and Wound Safety	7. Standard Precautions; 8. Clean, Aseptic and Sterile Principles; 9. Disposal and Exposure	6 questions
4	Equipment, Dressings and Basic Support	10. Equipment; 11. Dressing Categories; 12. Basic Dressing Support	6 questions
5	Common Wounds and Special Populations	13. Pressure Injuries; 14. Diabetic, Venous, Arterial and Skin-Tear Awareness; 15. Surgical Wounds and Burns	6 questions

6	Nutrition, Pain, Dignity and Communication	16. Nutrition and Hydration; 17. Pain and Emotional Impact; 18. Person-Centred Support	6 questions
7	Complications, Escalation, Documentation and Competency	19. Red Flags; 20. Documentation and Handover; 21. Integrated Practice and Verification	6 questions

9. Complete Module 1 — Foundations of Wounds and Healing

Lesson 1. What Is a Wound?

A wound is a break or damage in the normal structure of skin or underlying tissue. It may be intentional, such as a surgical incision, or unintentional, such as a cut, skin tear or pressure-related injury.

The caregiver's role is to protect, observe, support the authorised care plan and report change. The caregiver does not independently diagnose or prescribe.

Scenario: A family member asks a caregiver to choose an antibiotic cream. **Best response:** Do not prescribe or select treatment; refer to the authorised clinician.

Knowledge check: What is the online course allowed to teach? **Answer:** Wound-care knowledge, observation, safety, communication, documentation and escalation—not unauthorised clinical treatment.

Lesson 2. Wound Types

Acute wounds occur suddenly; chronic wounds persist or heal slowly. Wounds may be open or closed, surgical, traumatic, pressure-related, diabetic-related, venous, arterial, burns, ulcers or skin tears.

The type, location, age, circulation, infection, treatment, health status and nutrition affect appearance and healing. A caregiver should describe what is seen and compare it with previous records rather than assign a diagnosis.

Scenario: A wound near the ankle is dark and painful. **Best response:** Record observations and escalate; do not declare it arterial or infected.

Lesson 3. Healing and Delayed Healing

Healing is commonly described in phases: haemostasis, inflammation, proliferation and remodelling/maturation. These phases overlap and do not look identical in every wound.

Healing may be delayed by infection, poor circulation, diabetes, pressure, repeated trauma, smoking, poor nutrition, dehydration, certain medicines, age, immobility or other health conditions. Caregivers should notice risk and report it, not attempt to correct the underlying disease.

Scenario: A chronic wound looks unchanged over several reviews. **Best response:** Document the trend and seek professional review rather than assuming it is normal.

10. Complete Module 2 — Wound Observation and Assessment Awareness

Lesson 4. Observation Versus Diagnosis

Observation describes facts: “The surrounding area appears more red and swollen than yesterday.” Diagnosis interprets cause: “The wound is infected.” Caregivers should observe and report, not diagnose.

Use the client’s established care plan and documentation system. If a measurement is required, use the trained method and authorised equipment; depth should be assessed only by appropriately authorised personnel.

Scenario: The caregiver sees increased odour and drainage. **Best response:** Record the change and escalate promptly.

Lesson 5. What to Observe

Observe location, size or established measurements, wound edges, wound bed where appropriate, exudate/drainage, colour, odour, surrounding skin, redness, warmth, swelling, pain, bleeding and functional impact. Record changes over time.

Do not remove dressings, probe, swab, measure depth or expose a wound unless the care plan, training and authorisation permit it. Preserve privacy and comfort.

Lesson 6. Trends and Care Plans

A single observation is less informative than a reliable trend. Compare with previous documentation, care-plan instructions and the client’s report. A change in pain, drainage, odour, size, colour, function or general wellbeing may require review.

Scenario: A client says the dressing feels tighter and pain has increased. **Best response:** Do not adjust the dressing independently; protect comfort and escalate.

11. Complete Module 3 — Infection Prevention and Wound Safety

Lesson 7. Standard Precautions

Treat blood and body fluids as potentially infectious. Apply hand hygiene, appropriate PPE, single-use supplies, safe environmental preparation, contamination prevention and approved waste disposal. Follow the local IPC procedure; this course applies rather than reproduces NIC's broader Infection Prevention & Control CPD.

Scenario: A clean dressing touches an unclean surface. **Best response:** Treat it as contaminated and follow the replacement/disposal policy.

Lesson 8. Clean, Aseptic and Sterile Principles

“Clean” means reducing visible dirt and contamination; “aseptic” means preventing harmful contamination during a procedure; “sterile” means free from viable microorganisms under the applicable process. These terms are not interchangeable.

Which technique is required depends on wound, procedure, facility policy and clinician instruction. Caregivers must not downgrade a required aseptic or sterile process because supplies are inconvenient.

Lesson 9. Disposal and Exposure

Handle dressings, gauze, gloves and contaminated materials according to facility waste policy. Use sharps containers for sharps and never place needles or blades in ordinary waste. If blood/body-fluid exposure or needlestick occurs, provide immediate first aid, report urgently and follow occupational-health/emergency procedures.

Scenario: A used blade is found in a household bin. **Best response:** Do not reach into the bin; secure the area and report for safe handling.

12. Complete Module 4 — Equipment, Dressings and Basic Support

Lesson 10. Equipment

Common supplies include gloves, gauze, non-adherent materials, absorbent dressings, tape, bandages, cleansing solution, skin-protection products, waste bags, sharps containers and documentation. Exact items depend on the care plan and facility.

Check packaging, expiry, integrity, cleanliness, correct patient, correct wound and required equipment before support begins. Do not open supplies unnecessarily.

Lesson 11. Dressing Categories

Category	General characteristic	Safety boundary
Gauze	Basic absorbent or protective material	May adhere or shed fibres; use only as directed
Non-adherent	Designed to reduce sticking	Follow product and care-plan instructions
Absorbent/foam	Manages selected drainage	Not independently selected by caregiver
Transparent film	Protective visual cover in selected uses	Do not apply without authorised plan
Hydrocolloid	Occlusive dressing for selected wounds	Requires appropriate clinical selection
Alginate-type	Absorbs drainage in selected wounds	Requires authorised selection and correct use
Compression products	Apply pressure for selected conditions	Never independently initiate compression
Adhesives/bandages	Secures or supports dressings	Check skin and allergy risk
Cleansing solutions	Used according to wound protocol	Never substitute household products

The dressing used must follow the client's care plan or direction from an appropriately authorised healthcare professional.

Lesson 12. Basic Dressing Support

The conceptual sequence is:

Prepare → Identify → Explain → Consent → Hand Hygiene/PPE → Prepare Environment → Follow Care Plan → Support Dressing Procedure → Dispose Safely → Reassess/Observe → Document → Escalate.

Position safely, protect privacy, explain what is happening, be alert to pain, avoid unnecessary handling, maintain wound protection and dispose of contaminated materials appropriately. Where actual dressing changes are taught, they must be supervised and assessed.

13. Complete Module 5 — Common Wounds and Special Populations

Lesson 13. Pressure Injuries

Pressure injuries are associated with sustained pressure and factors such as immobility, moisture, friction, shear, poor nutrition, reduced sensation and poor circulation. Common sites include heels, sacrum, hips, elbows and other pressure points.

Caregiver actions include following the repositioning plan, checking skin as authorised, reducing pressure through approved equipment, managing moisture and reporting redness that persists, blistering, broken skin, colour change or pain. Do not stage or independently treat a pressure injury unless specifically trained and authorised.

Lesson 14. Diabetic, Venous, Arterial and Skin-Tear Awareness

Diabetes may impair sensation, circulation and healing. Foot observation should include skin integrity, colour change, temperature difference, swelling, drainage, cracks and footwear concerns; do not cut corns, remove calluses or treat infection independently.

Venous and arterial ulcers have different patterns, but caregivers should not diagnose from appearance. Skin tears are more common in fragile older skin; prevent friction and report separation, bleeding or deterioration. Professional review is required for new or worsening wounds.

Lesson 15. Surgical Wounds and Burns

Follow postoperative instructions exactly. Observe incision appearance, drainage, pain, swelling, separation and general illness, and report changes. Do not remove sutures, staples, drains or dressings unless authorised and trained.

Burn care is covered only at awareness level; first-aid emergencies belong in NIC's First Aid & Basic Life Support CPD. Report worsening pain, blistering, colour change, infection concern or any burn requiring professional assessment.

14. Complete Module 6 — Nutrition, Pain, Dignity and Communication

Lesson 16. Nutrition and Hydration

Protein, energy, fluids, vitamins, minerals and general nutritional status can influence healing. Caregivers may encourage appropriate food and drink within the authorised care plan and report poor intake, dehydration concerns, weight change or swallowing difficulty.

Do not prescribe diets, supplements or special nutrition. Link deeper learning to NIC' s Nutrition, Hydration & Mealtime Support CPD.

Lesson 17. Pain and Emotional Impact

Chronic wounds, odour, visible dressings, reduced mobility and dependence can affect confidence, sleep, social participation and mood. Ask about pain in a respectful way, observe non-verbal distress, use the authorised pain pathway and report change. Do not independently prescribe analgesia or alter medication.

Lesson 18. Person-Centred Support

Explain what will happen, obtain consent, protect privacy, cover the body, offer choices, respect culture and religion, use preferred language and maintain independence. Never shame a person for odour, drainage or appearance.

Scenario: A client refuses wound inspection after embarrassment. **Best response:** Pause, listen, offer privacy and choice, and escalate unresolved clinical risk without coercion.

15. Complete Module 7 — Complications, Escalation, Documentation and Competency

Lesson 19. Red Flags

Urgent concerns may include uncontrolled bleeding, rapidly increasing swelling, severe or increasing pain, significant wound separation, increasing redness/warmth/swelling, pus or concerning drainage, new odour, fever or systemic illness, blackened/necrotic-looking tissue, sudden colour change, loss of sensation, new functional impairment, signs of sepsis, suspected serious infection, unexpected bleeding, deterioration or failure to heal.

When concerned, escalate rather than improvise.

Lesson 20. Documentation and Handover

Record date/time, wound location, observation, established measurement where authorised, dressing/care provided according to plan, client response, pain, change from previous observation, escalation, person notified and follow-up instructions. Use objective language and protect confidentiality.

Lesson 21. Integrated Practice and Verification

Apply: identify the client and plan; explain and obtain consent; prepare safely; support only within authorisation; protect the wound; observe; escalate; document and hand over. Online knowledge does not establish clinical dressing competence.

16. Visual learning plan

Visual	Purpose
Healing-phase timeline	Explain haemostasis, inflammation, proliferation and remodelling
Observation-not-diagnosis comparison	Prevent overclaiming
Wound observation body map	Support location recording
Dressing-category matrix	Explain characteristics with local-plan warning
Clean/aseptic/sterile comparison	Clarify terms without oversimplifying
Pressure-point illustration	Support prevention awareness
Diabetic-foot observation guide	Encourage inspection and escalation
Stop-escalate card	Stop → Protect → Immediate authorised action → Escalate → Document
Documentation example	Compare objective and judgemental wording
Care pathway diagram	Connect wound care to NIC' s other CPDs

Visuals must avoid graphic or humiliating exposure, use diverse Nigerian adults and be reviewed by wound-care, infection-prevention and safeguarding leads.

17. Scenario-based learning

1. **Increased redness:** Describe and report; do not diagnose infection.
2. **Wrong dressing:** Do not substitute; follow plan and contact authorised professional.
3. **Needlestick:** Immediate first aid, urgent reporting and follow-up.
4. **Persistent pressure redness:** Reduce risk through authorised plan and escalate.
5. **Diabetic foot change:** Protect, avoid self-treatment and seek professional review.
6. **Wound odour:** Maintain dignity, observe objectively and escalate.
7. **Uncontrolled bleeding:** Follow First Aid & BLS/emergency pathway and notify clinician.
8. **Client refusal:** Respect, explore, offer privacy/choice and escalate risk.
9. **No supplies:** Do not improvise sterile/advanced care; escalate.
10. **Delayed healing:** Document trend and request review.

18. Module assessments

Module 1

1. Define a wound. **Damage or break in skin/tissue.**
2. Name four wound types. **Surgical, traumatic, pressure-related, diabetic-related, burn, ulcer or skin tear.**
3. What does a caregiver do? **Observe, support authorised plan and report.**
4. What should not happen? **Diagnose or prescribe.**
5. Name healing phases. **Haemostasis, inflammation, proliferation, remodelling.**
6. Name four delayed-healing factors. **Infection, diabetes, pressure, poor circulation, nutrition, dehydration or repeated trauma.**

Module 2

1. What is observation? **Objective description.**
2. What is diagnosis? **Clinical interpretation requiring appropriate professional authority.**
3. Name six observation fields. **Location, size, edges, drainage, colour, odour, surrounding skin, pain or bleeding.**
4. What if wound changes? **Compare, document and escalate.**
5. Should caregiver independently measure depth? **Only if authorised and trained.**

6. What should not happen? **Probe or expose unnecessarily.**

Module 3

1. What are standard precautions? **Measures treating blood/body fluids as potentially infectious.**
2. Are clean, aseptic and sterile identical? **No.**
3. What must be single-use where required? **Supplies such as dressings and instruments.**
4. What follows exposure? **Immediate first aid, urgent report and follow-up.**
5. Where do sharps go? **Approved sharps container.**
6. What should not happen? **Recap or place sharps in ordinary waste.**

Module 4

1. Who selects advanced products? **Authorised clinician according to plan.**
2. What is non-adherent dressing for? **Reducing sticking in selected uses.**
3. Can caregiver initiate compression? **Not without training and authorisation.**
4. State support sequence. **Prepare, identify, explain, consent, hygiene/PPE, plan, support, dispose, observe, document, escalate.**
5. What should not happen? **Use household products.**
6. What if supply is contaminated? **Do not use; replace/report.**

Module 5

1. Name pressure-injury risk factors. **Pressure, immobility, moisture, friction/shear, poor nutrition, reduced sensation or circulation.**
2. What should be reported? **Persistent redness, blistering, broken skin, colour change or pain.**
3. What is diabetic-foot awareness? **Inspection and prompt reporting, not treatment.**
4. What should not happen? **Cut corns or calluses independently.**
5. What should surgical care follow? **Discharge/care instructions.**
6. Are burns covered as a full first-aid course here? **No.**

Module 6

1. Name healing-related nutrition factors. **Protein, energy, fluids, vitamins/minerals**

and overall status.

2. Can caregiver prescribe supplements? **No.**
3. What emotional effects may occur? **Shame, anxiety, reduced confidence, isolation and distress.**
4. What should not happen? **Shame or ridicule.**
5. What does person-centred wound care include? **Consent, privacy, choice, dignity and culture.**
6. What if pain increases? **Observe, document and escalate.**

Module 7

1. Name six red flags. **Uncontrolled bleeding, increased pain, swelling, redness/warmth, pus, odour, fever, separation, colour change or deterioration.**
2. What is documentation? **Objective record of observations, care, response and escalation.**
3. Does documentation replace escalation? **No.**
4. What is the competency distinction? **Online knowledge versus supervised practical skill.**
5. What if concerned? **Escalate rather than improvise.**
6. What is the integrated sequence? **Identify, consent, prepare, support, observe, escalate, document, hand over.**

19. 40-question final assessment

Recommended pass mark: 80% (32/40). At least 30 questions are scenario/application based.

1. **MCQ:** A wound is: A) only a bruise; B) damage or break in tissue; C) a diagnosis; D) a prescription. **Answer: B.**
2. **Scenario:** Asked to prescribe wound cream. **Answer:** Do not prescribe; refer.
3. **True/False:** Online completion authorises clinical dressing procedures. **Answer: False.**
4. **Scenario:** Chronic wound unchanged. **Answer:** Document trend and seek review.
5. **Application:** Name healing phases. **Answer:** Haemostasis, inflammation, proliferation, remodelling.
6. **Scenario:** Dark painful ankle wound. **Answer:** Observe/report; do not diagnose.

7. **MCQ:** A caregiver's role is to: A) diagnose; B) observe, support plan and report; C) debride; D) prescribe. **Answer: B.**
8. **Scenario:** Family instruction conflicts with care plan. **Answer:** Follow plan and escalate conflict.
9. **MCQ:** Observation is: A) "appears redder than yesterday" ; B) "infected" ; C) "arterial" ; D) "necrotic diagnosis." **Answer: A.**
10. **Scenario:** Increased drainage. **Answer:** Record and escalate.
11. **True/False:** Depth should be assessed only by trained/authorised personnel. **Answer: True.**
12. **What should not happen?** Probe the wound to investigate.
13. **Scenario:** Dressing touches floor. **Answer:** Treat as contaminated and replace/report.
14. **MCQ:** Aseptic and sterile mean exactly the same. **Answer: False.**
15. **Scenario:** Used sharp in household bin. **Answer:** Do not reach in; report and secure safe handling.
16. **True/False:** Needlestick requires urgent reporting. **Answer: True.**
17. **Scenario:** No approved cleanser available. **Answer:** Do not improvise household product; escalate.
18. **MCQ:** Advanced dressing selection follows: A) caregiver preference; B) care plan/authorised clinician; C) social media; D) cheapest product. **Answer: B.**
19. **Scenario:** Caregiver asked to start compression. **Answer:** Decline unless trained/authorised under plan.
20. **Application:** State basic dressing-support sequence. **Answer:** Prepare, identify, explain, consent, hygiene/PPE, plan, support, dispose, observe, document, escalate.
21. **Scenario:** Persistent redness over heel. **Answer:** Follow prevention plan and escalate.
22. **MCQ:** Pressure risk includes: A) immobility and moisture; B) only age; C) good circulation; D) exercise. **Answer: A.**
23. **Scenario:** Diabetic foot has new colour change. **Answer:** Protect, report promptly, do not self-treat.
24. **What should not happen?** Cut callus independently.
25. **True/False:** Skin tears require prevention of friction and reporting. **Answer: True.**
26. **Scenario:** Surgical incision separates. **Answer:** Urgent professional escalation.
27. **MCQ:** Nutrition support should be: A) prescribed by caregiver; B) encouraged within plan and concerns reported; C) replaced with supplements automatically; D) ignored.

Answer: B.

28. **Scenario:** Increased wound pain and anxiety. **Answer:** Provide respectful support, document and escalate.
29. **True/False:** Odour can affect dignity and wellbeing. **Answer: True.**
30. **What should not happen?** Shame a client about drainage or odour.
31. **Scenario:** Pus, fever and worsening pain. **Answer:** Urgent escalation.
32. **MCQ:** The correct response to concern is: A) improvise; B) escalate; C) hide; D) wait indefinitely. **Answer: B.**
33. **Scenario:** Caregiver writes “bad wound.” **Answer:** Replace with objective observations.
34. **True/False:** Documentation replaces emergency escalation. **Answer: False.**
35. **Scenario:** Client refuses inspection. **Answer:** Respect, explore, offer choice/privacy and escalate unresolved risk.
36. **Scenario:** Bleeding is uncontrolled. **Answer:** Use First Aid/BLS/emergency pathway and notify clinician.
37. **MCQ:** Practical competence requires: A) online reading; B) supervised assessment; C) self-declaration; D) attendance. **Answer: B.**
38. **Scenario:** Learner cannot maintain aseptic process. **Answer:** Stop, remediate and do not sign off.
39. **What should not happen?** Select an advanced therapy from a product catalogue.
40. **Integrated scenario:** New wound odour, increased pain, poor intake and client embarrassment. **Answer:** Preserve dignity, observe objectively, support authorised plan, report nutrition/pain/change, escalate promptly and document.

20. Answer key

The answers are embedded in the assessment. Core reasoning is consistent: observe rather than diagnose, follow the care plan, use infection precautions, protect dignity, escalate red flags, and never treat online completion as authorisation for invasive or advanced wound procedures.

21. Practical competency verification framework

Practical verification must be separate from online completion and available only where the learner is authorised. Use an appropriately qualified assessor, current local protocol, suitable supplies, safe environment and documented remediation.

Station	Minimum capability
Identity, explanation and consent	Confirms client/plan, explains and obtains consent
Preparation	Hand hygiene, PPE, equipment and environment
Privacy and positioning	Maintains dignity and safe comfort
Basic dressing support	Follows authorised plan without contamination or unnecessary handling
Observation	Identifies and reports changes without diagnosis
Waste/sharps	Disposes safely according to policy
Complication response	Stops, protects, escalates and documents
Documentation	Records wound, care, response, notification and follow-up
Handover	Communicates concise factual information

Critical failures: performing without authorisation; diagnosing/prescribing; proceeding without consent; contaminating supplies; unsafe sharps handling; ignoring severe pain/bleeding/deterioration; applying unapproved advanced therapy; concealing a concern; or signing off an unobserved skill.

22. Downloadable resources

1. Wound Observation Quick Reference.
2. Wound Red Flags & Escalation Guide.
3. Basic Dressing Support Checklist.
4. Pressure Injury Prevention Checklist.
5. Diabetic Foot Observation Guide.
6. Wound Documentation Template.
7. Dressing & Equipment Reference Guide.
8. Practical Competency Checklist.
9. Client Communication Guide.

10. Stop–Protect–Escalate–Document card.

23. Certificate recommendation

Online CPD certificate

This certificate recognises successful completion of NIC online CPD in wound-care awareness and basic dressing support principles, including wound biology, wound types, observation, infection prevention, dressing categories, pressure-injury and diabetic-foot awareness, nutrition and hydration links, pain, dignity, documentation and escalation. **This certificate represents online knowledge completion only. It does not authorise diagnosis, prescribing, debridement, suturing, advanced wound therapy or clinical dressing procedures. It is not a nursing, medical or wound-care specialist qualification, professional licence or automatic statutory workplace certification.**

CPD plus practical verification endorsement

This endorsement recognises NIC online CPD and successful supervised verification of the specific authorised dressing-support skills listed in the assessor record on [date] by [assessor/provider]. It represents demonstrated competence only in those specified skills and does not constitute professional licensure or authority beyond the learner's role, employer policy and applicable requirements.

24. Digital badge recommendation

NIC Wound Care & Basic Dressing Support — Knowledge Badge

Field	Specification
Description	Online CPD knowledge in safe caregiver-level wound observation and support
Competencies	Healing awareness, observation, IPC, dressing categories, red flags, dignity, documentation and escalation
CPD hours	5–6 online hours
Evidence	Modules, assessments, final score $\geq 80\%$, scope acknowledgement
Limitation	No practical authorisation or clinical licence

Practical competency verified endorsement

Requires assessor identity/qualification, date, exact skills, equipment, protocol/version, checklist, result, remediation and reassessment date. It must not imply specialist wound-care qualification or statutory authority.

25. Portal implementation notes

Use **Course → Modules → Lessons → Knowledge Checks → Module Assessments → Final Assessment → Certificate**. Track online completion, score, scope acknowledgement, practical verification status, assessor evidence, badge status and review date separately.

26. Nigerian context and scope-of-practice notes

Hospitals, clinics, residential care, home care and community care differ in staffing, supplies, water, electricity, transport, referral access and clinical supervision. NIC should require each employer to insert current wound-care SOPs, waste procedures, referral contacts, dressing formularies, documentation system and authorised caregiver activities.

Do not invent Nigerian legal or regulatory duties. Where scope, delegation or reporting requirements are referenced, verify them against current authoritative Nigerian sources and employer policy before launch. International wound guidance does not replace local clinical governance.

27. References

- [1] World Health Organization, WHO guidance and tools for wound care and infection prevention resources.
- [2] NICE, Pressure ulcers: prevention and management.
- [3] IWGDF/IDSA, Guidelines on the diagnosis and treatment of diabetes-related foot infections.
- [4] International Wound Infection Institute, Wound Infection in Clinical Practice consensus update.
- [5] World Health Organization, WHO Guidelines on Hand Hygiene in Health Care.

28. Quality control review

Review area	Finding	Required action
Clinical accuracy	Healing, wound types, observation, complications and escalation are covered at caregiver level.	Wound-care clinician review before launch.
Patient safety	Stop rules, scope boundaries and red flags are prominent.	Add local escalation contacts.
Scope	Diagnosis, prescribing, debridement, suturing and advanced therapies are excluded.	Governance sign-off.
Infection prevention	IPC is applied to wound care without duplicating the broader CPD.	Confirm local waste and exposure SOP.
Nigerian relevance	Home, hospital, community and resource variation are acknowledged.	Insert verified facility pathways.
Assessments	40 questions with scenario emphasis are included.	Pilot and item-analyse.
Practical competency	Separate supervised framework and critical failures are provided.	Approve assessor qualifications.
Duplication	Links to IPC, First Aid, ADL, Mobility, Nutrition, Diabetes and Documentation CPDs are clear.	Add portal cross-links.
Accessibility	Plain language, tables, scenarios and job aids support varied learners.	Pilot with caregivers.
Visual quality	Visual plan supports learning without graphic or humiliating imagery.	Safeguarding review of visuals.

Final learner message: “I understand wounds, I can recognise important changes, I know how to support safe wound care within my role, and I know when to escalate.”